

SAME SIZE ARTWORK
LEAFLET SIZE : 264 mm x 186 mm

Blood

Uncommon: eosinophilia, leukopenia.

Rare: neutropenia, thrombocytopenia.

Very rare: agranulocytosis.

Isolated cases: haemolytic anaemia, pancytopenia.

Others

Uncommon: asthenia, fungal overgrowth and proliferation of other resistant microorganisms. Very rare: allergic pneumonitis, fever.

Other undesirable effects which have been associated with fluoroquinolone administration include:

- extrapyramidal symptoms and other disorders of muscular coordination,
- hypersensitivity vasculitis,
- attacks of porphyria in patients with porphyria.

Psychiatric disorders

Rare: Psychotic reactions (with e.g. hallucination, paranoia), Depression

Very Rare: Psychotic reactions (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)

Not known (cannot be estimated from available data): Psychotic disorders with selfendangering behavior including suicidal ideation or suicide attempt

PRECAUTIONS/WARNINGS:

The use of levofloxacin should be avoided in patients who have experienced serious adverse reactions in the past when using fluoroquinolones containing products (see section Adverse Effects/Undesirable Effects). Treatment of these patients with levofloxacin should only be initiated in the absence of alternative treatment options and after careful benefit/risk assessment.

Aortic aneurysm and dissection

Epidemiologic studies report an increased risk of aortic aneurysm and dissection after intake of fluoroquinolones, particularly in the older population. Therefore, fluoroquinolones should only be used after careful benefit-risk assessment and after consideration of other therapeutic options in patients with positive family history of aneurysm disease, or in patients diagnosed with pre-existing aortic aneurysm and/or aortic dissection, or in presence of other risk factors or conditions predisposing for aortic aneurysm and dissection (e.g. Marfan syndrome, vascular Ehlers-Danlos syndrome, Takayasu arteritis, giant cell arteritis, Behcet's disease, hypertension, known atherosclerosis).

In case of sudden abdominal, chest or back pain, patients should be advised to immediately consult a physician in an emergency department.

Prolonged, disabling and potentially irreversible serious adverse drug reactions Very rare cases of prolonged (continuing months or years), disabling and potentially irreversible serious adverse drug reactions affecting different, sometimes multiple body systems (musculoskeletal, nervous, psychiatric and senses) have been reported in patients receiving fluoroquinolones irrespective of their age and pre-existing risk factors. Levofloxacin should be discontinued immediately at the first signs or symptoms of any serious adverse reaction and patients should be advised to contact their prescriber for advice.

Tendinitis and tendon rupture

Tendinitis and tendon rupture (especially but not limited to Achilles tendon), sometimes bilateral, may occur as early as within 48 hours of starting treatment with fluoroquinolones and have been reported to occur even up to several months after discontinuation of treatment. The risk of tendinitis and tendon rupture is increased in older patients (above 60 years of age), with renal impairment, patients with solid organ transplants, and those treated concurrently with corticosteroids*. Therefore, concomitant use of corticosteroids should be avoided.

At the first sign of tendinitis (e.g. painful swelling, inflammation) the treatment with levofloxacin should be discontinued and alternative treatment should be considered. The affected limb(s) should be appropriately treated (e.g. immobilisation). Corticosteroids should not be used if signs of tendinopathy occur.

*[For systemically administered levofloxacin-containing products, the listing of risk factors should additionally include: "in patients receiving daily doses of 1000 mg levofloxacin".]

Peripheral neuropathy

Cases of sensory or sensorimotor polyneuropathy resulting in paraesthesia, hypaesthesia, dysesthesia, or weakness have been reported in patients receiving quinolones and fluoroquinolones. Patients under treatment with levofloxacin should be advised to inform their doctor and pharmacist prior to continuing treatment if symptoms of neuropathy such as pain, burning, tingling, numbness, or weakness develop in order to prevent the development of potentially irreversible condition (see section Adverse Effects/Undesirable Effects).

In the most severe cases of pneumococcal pneumonia Glevo may not be the optimal therapy. Nosocomial infections due to P. aeruginosa may require combination therapy.

Clostridium difficile-associated disease

Diarrhoea, particularly if severe, persistent and/or bloody, during or after treatment with Glevo tablets, may be symptomatic of Clostridium difficile-associated disease, the most severe form of which is pseudomembranous colitis. If pseudomembranous colitis is suspected, Glevo tablets must be stopped immediately and patients should be treated with supportive measures ± specific therapy without delay (e.g. oral vancomycin). Products inhibiting the peristalsis are contraindicated in this clinical situation.

Patients predisposed to seizures

Glevo tablets are contraindicated in patients with a history of epilepsy and, as with other quinolones, should be used with extreme caution in patients predisposed to seizures, such as patients with pre-existing central nervous system lesions, concomitant treatment with fenbufen and similar non-steroidal anti-inflammatory drugs or with drugs which lower the cerebral seizure threshold, such as theophylline

Patients with G-6-phosphate dehydrogenase deficiency

Patients with latent or actual defects in glucose-6-phosphate dehydrogenase activity may be prone to haemolytic reactions when treated with quinolone antibacterial agents, and so levofloxacin should be used with caution.

Patients with renal impairment

Since levofloxacin is excreted mainly by the kidneys, the dose of Glevo should be adjusted in patients with renal impairment.

Prevention of photosensitisation

Although photosensitisation is very rare with levofloxacin, it is recommended that patients should not expose themselves unnecessarily to strong sunlight or to artificial UV rays (e.g. sunray lamp, solarium), in order to prevent photosensitisation.

Patients treated with Vitamin K antagonists

Due to possible increase in coagulation tests (PT/INR) and/or bleeding in patients treated with Glevo in combination with a vitamin K antagonist (e.g. warfarin), coagulation tests should be monitored when these drugs are given concomitantly

Exacerbation of myasthenia gravis

Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with fluoroquinolones use in persons with myasthenia gravis. Avoid fluoroquinolones in patients with known history of myasthenia gravis.

Psychiatric reactions

Psychiatric reactions may occur even after the first administration of fluoroquinolones, including Glevo 250/500. In rare cases, depression or psychotic reactions can progress to suicidal ideations/thoughts and self-injurious behaviour, such as attempted or completed suicide (see section 'Undesirable effects'). In the event that the patient develops these reactions, Glevo 250/500 should be discontinued and appropriate measures instituted. Caution is recommended if Glevo 250/500 is to be used in psychotic patients or in patients with a history of psychiatric disease

DRUG INTERACTION

Iron salts, Magnesium- or Aluminium-containing antacids:

Levofloxacin absorption is significantly reduced when iron salts, or magnesium or aluminium containing antacids are administered with Levofloxacin tablets. It is recommended that preparations containing divalent or trivalent cations such as iron salts, or magnesium-or aluminium containing antacids should not be taken 2 hours before or after GLEVO tablets administration. No interaction was found with calcium carbonate.

Sucralfate

The bioavailability of Levofloxacin is significantly reduced when administered together with sucralfate. If the patient is to receive both sucralfate and GLEVO, it is best to administer sucralfate 2 hours after the GLEVO tablet administration.

Theophylline, fenbufen or similar non-steroidal anti-inflammatory drugs

No pharmacokinetic interactions of Levofloxacin were found with theophylline in a clinical study. However, a pronounced lowering of the cerebral seizure threshold may occur when quinolones are given concurrently with theophylline, nonsteroidal anti-inflammatory drugs, or other agents which lower the seizure threshold. Levofloxacin concentrations were about 13% higher in the presence of fenbufen than when administered alone.

Probenecid and Cimetidine

Probenecid and cimetidine had a statistically significant effect on the elimination of Levofloxacin. The renal clearance of Levofloxacin was reduced by cimetidine (24%) and probenecid (34%). This is because both drugs are capable of blocking the renal tubular secretion of Levofloxacin. However, at the tested doses in the study, the statistically significant kinetic differences are unlikely to be of clinical relevance.

Caution should be exercised when Levofloxacin is coadministered with drugs that affect the tubular renal secretion such as probenecid and cimetidine, especially in renally impaired patients.

Cyclosporin

The half life of cyclosporin was increased by 33% when coadministered with Levofloxacin.

Antidiabetic agents

Disturbances of blood glucose, including hyperglycemia and hypoglycemia, have been reported in patients treated concomitantly with quinolones and an antidiabetic agent. Therefore, careful monitoring of blood glucose is recommended when these agents are co-administered.

Vitamin K antagonists

Increased coagulation tests (PT/INR) and/or bleeding, which may be severe, have been reported in patients treated with levofloxacin in combination with a vitamin K antagonist (e.g. warfarin). Coagulation tests, therefore, should be monitored in patients treated with vitamin K antagonists

Other drug interactions

Levofloxacin pharmacokinetics was not affected to a clinically relevant extent when Levofloxacin was coadministered with the following drugs: digoxin, glibenclamide, ranitidine, and warfarin.

RECOMMENDED DOSAGE , DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION :

Following are the dosage recommendations for GLEVO:

Dosage in patients with Normal Renal Function (Creatinine clearance >50 ml/min):

Indication	Daily dose regimen (accordingly to severity)	Duration of treatment
Acute Sinusitis	500 mg once daily	10-14 days
Acute Exacerbation of Chronic Bronchitis	500 mg once daily	7-10 days
Community-Acquired Pneumonia	500 mg once daily	7-14 days
Uncomplicated skin and soft tissue infections	500 mg once daily	7-10 days
Uncomplicated Urinary Tract Infections (UTIs)	250 mg once daily	3 days
Complicated Urinary Tract Infections in cluding Acute Pyelonephritis	250 mg once daily	10 days
Chronic Bacterial Prostatitis	500 mg once daily	28 days

Duration of treatment

The duration of therapy varies according to the course of the disease with a maximum duration of treatment of 14 days. As with antibiotic therapy in general, administration of GLEVO tablets should be continued for a minimum of 48 to 72 hours after the patient has become afebrile or evidence of bacterial eradication has been obtained.

Method of administration

GLEVO tablets should be swallowed without crushing and with sufficient amount of liquid. They may be divided at the score line to adapt the dosage. The tablets may be taken during meals or between meals.

Dosage in patients with Impaired Renal Function (Creatinine clearance <50 ml/min): Since Levofloxacin is excreted mainly by the kidneys, the dose of GLEVO should be adjusted in patients with renal impairment.

Indication	Creatinine Clearance		Haemodialysis	Chronic Ambulatory Peritoneal Dialysis (CAPD)
	20 - 49 ml/min	10 - 19 ml /min		
Acute Exacerbation of Chronic Bronchitis / Community-Acquired Pneumonia / Acute Sinusitis / Uncomplicated skin and soft tissue infections	500 mg initially followed by 250 mg every 24 hours	500 mg initially followed by 250 mg every 48 hours	500 mg initially followed by 250 mg every 48 hours	500 mg initially followed by 250 mg every 48 hours
Complicated UTI / Acute Pyelonephritis	No dosage adjustment required	250 mg initially followed by 250 mg every 48 hours		
Uncomplicated UTI	No dosage adjustment required			

Dosage in patients with Impaired Liver Function

No dosage adjustment is required since Levofloxacin is not metabolised to any relevant extent by the liver and is mainly excreted by the kidneys.

Dosage in Elderly

No dosage adjustment is required in the elderly, other than that imposed by consideration of renal function.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE (S) :

The most important signs to be expected following acute overdosage of Glevo tablets are central nervous system symptoms such as confusion, dizziness, impairment of consciousness, and convulsive seizures, increases in QT interval as well as gastro-intestinal reactions such as nausea and mucosal erosions.

In the event of overdose, symptomatic treatment should be implemented. ECG monitoring should be undertaken, because of the possibility of QT interval prolongation. Antacids may be used for protection of gastric mucosa. Haemodialysis, including peritoneal dialysis and CAPD, are not effective in removing levofloxacin from the body. No specific antidote exists.

PACKING / PACK SIZES :

Glevo 250 Tablets : Strip of 5 tablets .

Glevo 500 Tablets : Strip of 5 tablets .

SHELF LIFE : 2 Years .

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS

Store below 30°C. Protect from moisture & light.

Glevo 250 Tablets MAL 06030806AZ

Glevo 500 Tablets MAL 06030807AZ

Revision Date: 31 Jul 2024

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glenmark
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ICONGRAPHICS CODE:

PANTONE SHADE



PANTONE
BLACK
PROCESS C

**Supersedes
Artwork Code
PE37786**

PHARMACODE: